

Imagine you have a medical condition that requires surgery. It's a bit tricky, but the procedure has a good chance of success. You interview a few doctors, looking at their academic history, published papers, experience, and reputations. You narrow the list to two surgeons and get access to their surgical records, including patient survival rates.

Both surgeons have very good reputations; one works primarily for private patients covered by insurance, the other is at a top medical school. The private doctor runs a success/survival rate of 86%, while the medical school doc runs at 61%.

Which doctor do you choose?

If you immediately said the 86% doctor, you are outcome focused. You saw the better results and that was all you needed to know. World Series of Poker Tournament Champion Annie Duke, in her 2018 book *Thinking in Bets*, calls this “resulting.”³⁰² This is the cognitive error of evaluating decisions based solely on their outcomes, rather than on the quality of the decision-making process behind them.

Instead of “resulting,” you might have wondered why a cutter with a great reputation at a top-ranked medical school had a much worse survival ratio. You do a little more research into her process. You find that she invented this procedure 30 years ago. She did all of the early experimental surgeries, including lots of clinical failures (meaning bad surgical outcomes and low survival rates). But over time, she refined the surgery, where through trial and error she developed what is now a life-saving technique. Indeed, her methodology has become the standard, thanks to her research. Every doctor and patient who followed afterward benefited from her groundbreaking clinical work.³⁰³

Because of her background in this area, this doc gets all of the “impossible” surgical cases. When other surgeons don't think they can do the operation—or don't want to negatively impact their batting average—they refer it to her medical school. Hopeless and complicated surgeries make up a big part of